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FREE GUIDE

TRIMESTER-BY-TRIMESTER FRAMEWORK

Pregnancy Weight Gain Control System

Nutrition targets, meal structures and a weekly check-in system — compiled from IOM, ACOG, NIH, USDA and FDA published guidance.

03	08	40	06
TRIMESTERS MAPPED	KEY NUTRIENTS WITH TARGETS	WEEK TRACKING SHEET	NAMED SOURCES

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Informational guide — not medical advice

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Who this is written for

Expectant mothers looking for a structured way to follow the weight-gain ranges and nutrient targets published by the named authorities. It is written for uncomplicated singleton pregnancies and is not a substitute for individual clinical care — your own clinician's guidance always takes precedence. A varied and balanced diet and a healthy lifestyle are important.

01 How to Use This System

This system has three parts: **track**, **nourish**, and **adjust**. You track your weight week by week, nourish your body with the right nutrients at the right time, and adjust your intake based on how your pregnancy is progressing — not on a rigid diet.

1. Find your starting point

Use the table in **Section 02** to find your recommended total weight gain based on your pre-pregnancy BMI. This is your target range. [1][2]

2. Track weekly

Weigh yourself once a week, same day, same time, same scale. Record it in the tracker in **Section 09**. Small fluctuations are normal — look at the trend, not the daily number.

3. Nourish by trimester

Each trimester has different nutritional priorities. Use the nutrition board in **Section 04** and the meal frameworks in **Section 05** to guide your choices.

4. Adjust as needed

If you're gaining too fast or too slow, adjust by 100–200 calories per day and recheck in two weeks. If you're consistently outside your range, talk to your clinician.

> **Important**
>
> This system is based on IOM/ACOG guidelines. It is **not medical advice**. Always consult your own clinician before making changes to your diet or exercise routine. Individual recommendations vary based on your health, your baby's growth, and your clinician's guidance. [1][2]

02 Weight Gain by Pre-Pregnancy BMI

The Institute of Medicine (IOM) and ACOG recommend the following total weight gain ranges based on your pre-pregnancy body mass index (BMI). [1][2]

Pre-Pregnancy BMI	Recommended Total Gain	2nd/3rd Trimester Rate
Underweight (< 18.5)	28–40 lbs (12.5–18 kg)	1–1.3 lbs/week
Normal (18.5–24.9)	25–35 lbs (11.5–16 kg)	0.8–1 lb/week
Overweight (25–29.9)	15–25 lbs (7–11.5 kg)	0.5–0.7 lbs/week
Obese (30+)	11–20 lbs (5–9 kg)	0.4–0.6 lbs/week

[1] Institute of Medicine. *Weight Gain During Pregnancy: Reexamining the Guidelines*. 2009. [2] ACOG Committee Opinion 548 (reaffirmed 2020).

Where Does the Weight Go?

In a typical pregnancy, weight gain is distributed roughly as follows: [1]

Component	Weight	Component	Weight
Baby	7–8 lbs (3.2–3.6 kg)	Increased blood volume	3–4 lbs (1.4–1.8 kg)
Placenta	1–2 lbs (0.5–0.9 kg)	Increased fluid volume	2–3 lbs (0.9–1.4 kg)
Amniotic fluid	2 lbs (0.9 kg)	Uterus	2 lbs (0.9 kg)
Breast tissue	1–3 lbs (0.5–1.4 kg)	Maternal fat stores	6–8 lbs (2.7–3.6 kg)

Ranges are approximate and vary between individuals. [1]

03 Weight Gain by Trimester

First Trimester (Weeks 1–13)

Most women gain 1–5 pounds total in the first trimester. If you have significant nausea or vomiting, you may lose weight initially. This is usually not a concern if you are able to stay hydrated and eat some calories. [1]

Second Trimester (Weeks 14–27)

Weight gain typically accelerates in the second trimester as nausea fades and appetite returns. Most women gain about 1 pound per week. [1][2]

Third Trimester (Weeks 28–40)

Weight gain continues at about 1 pound per week, though some women gain more in the final weeks. Growth may slow or plateau in the last 2–3 weeks. [1]

> **"Eating for two" is a myth**
>
> You only need about **300 extra calories per day** in the second trimester and **450** in the third. That's roughly a snack, not a full extra meal. [1]

Calorie Needs by Trimester

Trimester	Extra Calories/Day	Focus
1st (weeks 1–13)	0 extra	Manage nausea, stay hydrated, prenatal vitamin
2nd (weeks 14–27)	+300	Protein + iron for baby's growth spurt
3rd (weeks 28–40)	+450	DHA + calcium for brain and bone development

[1] Institute of Medicine. *Weight Gain During Pregnancy: Reexamining the Guidelines*. 2009.

04 Nutrition Board — Daily Targets

These are the key nutrients for pregnancy, with food sources and daily targets. Focus on food first; supplements fill gaps, not replace meals. [3][4][5]

Nutrient	Daily Target	Key Food Sources
Folate	600 mcg	Lentils, spinach, asparagus, fortified cereals
Iron	27 mg	Lean red meat, lentils, spinach, fortified oats
DHA	200–300 mg	Salmon, sardines, walnuts, chia seeds
Choline	450 mg	Eggs, lean beef, salmon, broccoli
Calcium	1,000 mg	Yogurt, milk, fortified plant milk, cheese
Vitamin D	600 IU	Fortified milk, salmon, egg yolks, sunlight
Protein	71 g	Chicken, eggs, Greek yogurt, lentils, tofu
Fiber	28 g	Oats, berries, beans, whole grains, vegetables

[3] NIH Office of Dietary Supplements. [4] USDA Dietary Guidelines for Americans 2020–2025. [5] ACOG Committee Opinion 548.

These are general targets

Your clinician may recommend different amounts based on your blood work, dietary restrictions, or pregnancy complications. Where authorities disagree, this guide follows the IOM/ACOG range and notes the source beside each figure. [1][3]

05 Trimester Meal Frameworks

Sample day structures — not rigid meal plans. Adjust portions based on your hunger, activity level, and clinician's guidance.

First Trimester (Weeks 1–13)

Focus: managing nausea, staying hydrated, getting folate. Eat small, frequent meals. Cold foods often smell less and are easier to tolerate.

SAMPLE DAY

Morning	Dry toast or crackers before getting up; ginger tea
Breakfast	Fortified oatmeal with banana and walnuts
Snack	Greek yogurt with berries
Lunch	Lentil soup with whole-grain bread
Snack	Apple slices with peanut butter
Dinner	Baked chicken, steamed broccoli, white rice
Evening	Warm milk or fortified plant milk

Second Trimester (Weeks 14–27)

Focus: protein and iron for baby's growth spurt. Nausea usually fades — appetite returns. Add ~300 calories per day.

SAMPLE DAY

Breakfast	Scrambled eggs with spinach, whole-grain toast, orange
Snack	Trail mix: almonds, dried apricots, dark chocolate chips
Lunch	Quinoa bowl with chickpeas, roasted vegetables, tahini
Snack	Hummus with carrot and cucumber sticks
Dinner	Grilled salmon, sweet potato, steamed green beans
Evening	Cottage cheese with pineapple

Third Trimester (Weeks 28–40)

Focus: DHA for brain development, calcium for bones. Baby is pressing on your stomach — smaller, more frequent meals work better. Add ~450 calories.

SAMPLE DAY

Breakfast	Overnight oats with chia seeds, blueberries, honey
Snack	Hard-boiled egg + whole-grain crackers
Lunch	Turkey and avocado wrap with side salad
Snack	Smoothie: banana, spinach, Greek yogurt, flaxseed
Dinner	Lean beef stir-fry with brown rice and bok choy
Evening	Warm milk with turmeric (golden milk)

06 Portion Guide — What Does 300 Extra Calories Look Like?

Visual references for adding the right amount of calories in the 2nd and 3rd trimesters. [1][4]

Second Trimester (+300 kcal)

Pick one of these options to add to your normal day:

- Greek yogurt (170g) + 1 tbsp honey + 2 tbsp granola
- 2 slices whole-grain toast + 1 tbsp peanut butter + 1 banana
- Handful of almonds (30g) + 1 apple + 1 string cheese
- Smoothie: 1 cup milk + 1 banana + 2 tbsp oats + 1 tbsp flaxseed

Third Trimester (+450 kcal)

Pick one of these options to add to your normal day:

- Avocado toast: 2 slices whole-grain bread + ½ avocado + 1 poached egg
- Trail mix: 40g mixed nuts + 30g dried fruit + 20g dark chocolate
- Overnight oats: ½ cup oats + 1 cup milk + 1 tbsp chia + 1 tbsp almond butter
- Cheese & crackers: 40g cheese + 8 whole-grain crackers + 1 pear

> **Remember**
>
> These are additions to your normal meals, not replacements. If you're not hungry, don't force it — focus on nutrient density instead. [1]

Snack Ideas by Trimester

First Trimester — gentle on nausea	Second — energy boost	Third — compact nutrition
Ginger tea with whole-grain crackers	Trail mix: almonds, dried apricots, dark chocolate	Hard-boiled egg with whole-grain crackers
Plain yogurt with honey	Hummus with vegetable sticks	Smoothie: banana, spinach, Greek yogurt, flaxseed
Banana with almond butter	Apple slices with peanut butter	Avocado on whole-grain toast
Fortified cereal with milk	Cottage cheese with pineapple	Cheese with whole-grain crackers

[4] USDA Dietary Guidelines for Americans 2020–2025.

07 Common Questions

What if I'm gaining too fast?

First, check your trend over two weeks — not a single day. If you are consistently above your weekly target, reduce by about 100 calories per day. Prioritize protein, fiber, and water over calorie-dense liquids. If the trend continues, talk to your clinician. [1][2]

What if I'm gaining too slowly?

Check whether you are meeting your food group targets, especially protein, calories, and hydration. Add a nutrient-dense snack rather than eating large portions of low-nutrient food. Appetite changes are common; discuss persistent under-gain with your clinician. [1]

Do I need to follow this exactly?

No. This is a guide, not a prescription. Your body, health history, nausea, appetite, blood work, and pregnancy progression all affect the right targets for you. Use the framework and adjust with your clinician. [2]

Can I exercise?

For most uncomplicated pregnancies, moderate exercise is safe and beneficial. Walking, swimming, stationary cycling, and prenatal yoga are common options. Follow your clinician's guidance and stop for bleeding, dizziness, pain, or reduced fetal movement. [2]

08 Foods to Limit or Avoid

These foods carry elevated risk during pregnancy. [1][2]

Avoid	Why
Raw or undercooked fish/meat	Risk of listeria, toxoplasmosis, salmonella
High-mercury fish (shark, swordfish, king mackerel)	Mercury harms baby's brain development
Unpasteurized dairy and soft cheeses	Risk of listeria
Deli meats (unless heated to steaming)	Risk of listeria
Alcohol	No safe amount during pregnancy
Excess caffeine (> 200 mg/day)	May increase miscarriage risk
Raw sprouts	Risk of salmonella and E. coli

[1] ACOG. [2] FDA *Advice About Eating Fish*. 2024.

Food Lists — What to Prioritize and Limit

Prioritize daily	Examples	Why it matters
Protein	Chicken, eggs, Greek yogurt, lentils, tofu	Baby's tissue development; maternal blood volume
Iron-rich	Lean beef, spinach, fortified oats, lentils	Contributes to normal red blood cell formation
Calcium	Yogurt, milk, fortified plant milk, cheese	Baby's bone and teeth formation
Folate	Lentils, asparagus, fortified cereals, spinach	Contributes to maternal tissue growth in pregnancy
Omega-3	Salmon, sardines, walnuts, chia seeds	Baby's brain and eye development
Fiber	Oats, berries, beans, whole grains, vegetables	Supports normal digestion

[1] ACOG. [2] Institute of Medicine. [4] USDA Dietary Guidelines.

09 Weekly Check-In Tracker

Use this tracker every week. Record your weight, how you're feeling, and any adjustments you're making. Look at the trend — not the daily number.

Week	Date	Weight (lbs)	Weekly Gain	Notes
1				
2				
3				
4				
5				
6				
7				
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10				
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19				
20				

Check-In Prompts

- How was my energy this week?
- Did I eat protein at every meal?
- Did I take my prenatal vitamin daily?
- How is my digestion? (constipation is common in pregnancy)
- Am I staying hydrated? (aim for 8–10 glasses of water)
- Do I need to adjust my calorie intake based on this week's trend?

09 Weekly Check-In Tracker — continued

Weeks 21–40.

Week	Date	Weight (lbs)	Weekly Gain	Notes
21				
22				
23				
24				
25				
26				
27				
28				
29				
30				
31				
32				
33				
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When to call your clinician

You are losing weight in the first trimester (beyond normal nausea) · You are gaining weight too quickly or too slowly · You have concerns about your weight or nutrition · You have a history of an eating disorder.

How to read your own trend

Compare each week with the week before, not with your starting weight. A flat week after a bigger gain is normal. What matters is whether the four-week average sits inside the rate for your BMI band in Section 02. [1] [2]

10 Healthy Weight Gain Tips

Eat nutrient-dense foods

Choose foods rich in vitamins, minerals, and protein over empty calories. Every calorie should work for you and your baby. [1]

Stay hydrated

Drink plenty of water. Thirst is sometimes mistaken for hunger. Aim for 8–10 glasses per day. [1]

Exercise regularly

Moderate exercise during pregnancy is safe for most women and can help manage weight gain. Walking, swimming, and prenatal yoga are excellent choices. [2]

Listen to your body

Eat when you are hungry and stop when you are full. Pregnancy hunger is real — don't restrict calories without medical guidance. [1]

Avoid "eating for two"

You only need about 300 extra calories per day in the second trimester and 450 in the third. That's roughly a snack, not a full extra meal. [1]

> **Key principle**
>
> Weight gain is not linear. Some weeks you may gain more, some weeks less. Focus on the overall trend across weeks, not day-to-day changes. [1][2]

11 Food Lists — Prioritize and Limit

Prioritize Daily

Category	Examples	Why It Matters
Protein	Chicken, eggs, Greek yogurt, lentils, tofu	Baby's tissue development; maternal blood volume
Iron-rich	Lean beef, spinach, fortified oats, lentils	Contributes to normal red blood cell formation
Calcium	Yogurt, milk, fortified plant milk, cheese	Baby's bone and teeth formation
Folate	Lentils, asparagus, fortified cereals, spinach	Contributes to maternal tissue growth in pregnancy
Omega-3	Salmon, sardines, walnuts, chia seeds	Baby's brain and eye development
Fiber	Oats, berries, beans, whole grains, vegetables	Supports normal digestion

Limit

Food	Reason to Limit
Caffeine	Keep under 200 mg per day (about one 12 oz coffee)
Added sugars	Empty calories that displace nutrient-dense foods
Processed meats	High sodium; preservatives offer no benefit
Unpasteurized dairy	Risk of listeria
Alcohol	No safe amount during pregnancy
High-mercury fish	Shark, swordfish, king mackerel, tilefish

[1] ACOG. [2] Institute of Medicine. [4] USDA Dietary Guidelines.

12 Disclaimer & Sources

Disclaimer

Not medical advice. This guide is for informational and educational purposes only. It does not replace professional medical advice, diagnosis, or treatment. Always consult your own clinician. Individual weight gain recommendations vary based on your health, your baby's growth, and your clinician's guidance. No outcome is guaranteed. [1][2]

Target Population

Written for expectant mothers with uncomplicated singleton pregnancies. It is not intended for multiple pregnancies, or for anyone with a medical condition affecting weight, fluid balance or nutrition, who should follow individual clinical advice.

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References

1. Institute of Medicine. *Weight Gain During Pregnancy: Reexamining the Guidelines*. Washington, DC: National Academies Press; 2009.
2. American College of Obstetricians and Gynecologists. Committee Opinion 548: Weight Gain During Pregnancy (reaffirmed 2020).
3. NIH Office of Dietary Supplements. *Dietary Supplement Fact Sheets*. ods.od.nih.gov.
4. USDA. *Dietary Guidelines for Americans 2020–2025*. dietaryguidelines.gov.
5. FDA. *Advice About Eating Fish*. 2024. fda.gov.
6. ACOG. *Nutrition During Pregnancy*. acog.org.

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